

AM INDO-AMERICAN L & RESEARCH INSTITUTE

Meri Basavataraka Ramarao Memorial Cancer Foundation
& Indo-American Cancer Organisation
10, Banjara Hills, Hyderabad 500034, India
55 1235/2355 6655, Fax : 91-40-2354 2120
Induscancer.com www.induscancer.com



MRN No : MR2320948

Age/Gender : 57 Years and 5 Months / M

Category : CS

...neous hypoechoic lesion noted in the
...wards left side. Lesion show mild
...abscess.

04.2024 14:37



**BASAVATARAKAM INDO-AMERICAN
CANCER HOSPITAL & RESEARCH INSTITUTE**

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& Indo-American Cancer Organisation

Road No 10, Banjara Hills, Hyderabad 500034, India

Phones : 91-40-2355 1235/2355 6655, Fax : 91-40-2354 2120

E-Mail : info@induscancer.com www.induscancer.com



Rad Accession No : RDB27816068

MRN No : MR2320948

Name : Mohanty Rama Chandra

Age/Gender : 57 Years and 5 Months / M

Consultant :

Category : CS

U/S PERIANAL of 29.04.2024:

Carcinoma Rectum, s/p APR

Evidence of 30 x 20 mm size ill-defined heterogeneous hypoechoic lesion noted in the perineum (perianal region), posteriorly more towards left side. Lesion show mild increased peripheral vascularity – likely evolving abscess.

Significant perilesional fat stranding seen.

Advice: MRI correlation.

Dr Harish Kacharagadla
Radiologist

Report Approved/Verified Date & Time

30.04.2024 14:37

**INDO-AMERICAN
CANCER HOSPITAL & RESEARCH INSTITUTE**



MC-2050

L&D Information System

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BASAVATARAKAM

INDO-AMERICAN
CANCER HOSPITAL & RESEARCH INSTITUTE

Patient ID	: MR2320948	Reference doctor	: Dr. SENTHIL J RAJAPPA
Patient name	: Mr. RAMA CHANDRA MOHANTY	Category	: CASH
Age / Gender	: 57 Y / Male	Sample collected	: 26-Apr-2024 10:02:16 AM
Visit ID	: HCMR240080579	Sample acknowledged	: 26-Apr-2024 10:40:16 AM
Bed number	:	Reported on	: 26-Apr-2024 12:01:20 PM
Sample no	:	Sample type	: Serum

BIOCHEMISTRY DEPT

TEST NAME (METHOD)	RESULTS	BIOLOGICAL REFERENCE INTERVAL	UNITS
SERUM CREATININE (ENZYMATIC SARCOSINE)	0.51	0.66 - 1.25	mg/dl

-----: End of Report -----

26-Apr-2024 12:01 PM

Dr. Sanjay BagadeMBBS, MD (Biochemistry)
Sr. Consultant, Biochemistry
Regn.No:84888

Note: Please correlate clinically. Kindly discuss if necessary.

Typed By: 06095 26-Apr-2024 11:04:55 AM

This report relates to the specimen / material submitted to the laboratory



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Patient ID	: MR2320948	Reference doctor	: Dr. SENTHIL J RAJAPPA
Patient name	: Mr. RAMA CHANDRA MOHANTY	Category	: CASH
Age / Gender	: 57 Y / Male	Sample collected	: 26-Apr-2024 10:02:16 AM
Visit ID	: HCMR240080579	Sample acknowledged	: 26-Apr-2024 10:40:15 AM
Bed number	:	Reported on	: 26-Apr-2024 11:50:11 AM
Sample no	:	Sample type	: Serum

BIOCHEMISTRY DEPT

TEST NAME (METHOD)	RESULTS	BIOLOGICAL REFERENCE INTERVAL	UNITS
SERUM BILIRUBIN TOTAL (MODIFIED DIAZO)	0.8	0.20 - 1.30	mg/dl

-----: End of Report -----

26-Apr-2024 11:50 AM

Dr. Sanjay Bagade

MBBS, MD (Biochemistry)
Sr. Consultant, Biochemistry
Regn No: 84888

NOTE: Please correlate clinically. Kindly discuss if necessary.

Typed By: DCMYS : 26-Apr-2024 11:04:39 AM

This report relates to the specimen / material submitted to the laboratory



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Patient ID	: MR2320948	Reference doctor	: Dr. SENTHIL J RAJAPPA
Patient name	: Mr. RAMA CHANDRA MOHANTY	Category	: CASH
Age / Gender	: 57 Y / Male	Sample collected	: 26-Apr-2024 10:02:16 AM
Visit ID	: HCMR240080579	Sample acknowledged	: 26-Apr-2024 11:07:30 AM
Bed number	:	Reported on	: 26-Apr-2024 04:20:58 PM
Sample no	:	Sample type	: Blood

HAEMATOLOGY

COMPLETE BLOOD PICTURE PLATELET COUNT

TEST NAME (METHOD)	RESULTS	BIOLOGICAL REFERENCE INTERVAL	UNITS
HAEMOGLOBIN (SLS-HAEMOGLOBIN METHOD)	81 ✓	130.00 - 170.00	g/l
PACKED CELL VOLUME (CUMULATIVE RBC PULSE HEIGHT METHOD)	0.24	0.36 - 0.46	l/l
RED CELL COUNT (HYDRO-DYNAMIC FOCUSING IMPEDENCE METHOD)	2.27	4.50 - 5.50	10(12)/l
WBC COUNT (HYDRO-DYNAMIC FOCUSING IMPEDENCE METHOD)	7.00 ✓	4.00 - 10.00	10 (9)/l
+FLUORESCENCE FLOW CYTOMETRY)			
NO OF CELLS COUNTED	100	-	
NEUTROPHILS (FLUORESCENCE FLOW CYTOMETRY /MICROSCOPY)	43	40.00 - 80.00	%
LYMPHOCYTE (FLUORESCENCE FLOW CYTOMETRY /MICROSCOPY)	27	20.00 - 40.00	%
MONOCYTE (FLUORESCENCE FLOW CYTOMETRY /MICROSCOPY)	24	2.00 - 6.00	%
EOSINOPHIL (FLUORESCENCE FLOW CYTOMETRY /MICROSCOPY)	06	1.00 - 6.00	%
BASOPHIL (FLUORESCENCE FLOW CYTOMETRY /MICROSCOPY)		-	%
BAND FORMS (FLUORESCENCE FLOW CYTOMETRY /MICROSCOPY)		-	%
METAMYELOCYTE (FLUORESCENCE FLOW CYTOMETRY /MICROSCOPY)		-	%
MYELOCYTE (FLUORESCENCE FLOW CYTOMETRY /MICROSCOPY)		-	%
PROMYELOCYTE (FLUORESCENCE FLOW CYTOMETRY /MICROSCOPY)		-	%
BLAST (FLUORESCENCE FLOW CYTOMETRY /MICROSCOPY)		-	%
ATYPICAL CELL		-	%
PLATELET COUNT (HYDRO-DYNAMIC FOCUSING IMPEDENCE METHOD/MICROSCOPY)	271 ✓	150.00 - 410.00	10 (9)/l
PERIPHERAL SMEAR	RBCs are normocytic normochromic. WBCs show normal count. Platelets are adequate.		

----- End of Report -----

26-Apr-2024 16:20 PM

Dr., KHALID ABDUL MANNAN

MD Pathology, DipRCPath

Regn.No.21877



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Patient ID	: MR2320948	Reference doctor	: Dr. SENTHIL J RAJAPPA
Patient name	: Mr. RAMA CHANDRA MOHANTY	Category	: CASH
Age / Gender	: 57 Y / Male	Sample collected	: 04-Apr-2024 04:33:42 PM
Visit ID	: HCMR240066183	Sample acknowledged	: 04-Apr-2024 05:31:56 PM
Bed number	:	Reported on	: 04-Apr-2024 07:43:09 PM
Sample no	:	Sample type	: Blood

HAEMATOLOGY

COMPLETE BLOOD PICTURE PLATELET COUNT

TEST NAME (METHOD)	RESULTS	BIOLOGICAL REFERENCE INTERVAL	UNITS
HAEMOGLOBIN (SLS-HEMOGLOBIN METHOD)	80	130.00 - 170.00	g/l
PACKED CELL VOLUME (CUMULATIVE RBC PULSE HEIGHT METHOD)	0.23	0.36 - 0.46	l/l
RED CELL COUNT (HYDRO-DYNAMIC FOCUSING IMPEDENCE METHOD)	2.25	4.50 - 5.50	10(12)/l
WBC COUNT (HYDRO-DYNAMIC FOCUSING IMPEDENCE METHOD + FLUORESCENCE FLOW CYTOMETRY)	5.67	4.00 - 10.00	10 (9)/l
NO OF CELLS COUNTED	100	-	-
NEUTROPHILS (FLUORESCENCE FLOW CYTOMETRY / MICROSCOPY)	51	40.00 - 80.00	%
LYMPHOCYTE (FLUORESCENCE FLOW CYTOMETRY / MICROSCOPY)	28	20.00 - 40.00	%
MONOCYTE (FLUORESCENCE FLOW CYTOMETRY / MICROSCOPY)	17	2.00 - 6.00	%
EOSINOPHIL (FLUORESCENCE FLOW CYTOMETRY / MICROSCOPY)	04	1.00 - 6.00	%
BASOPHIL (FLUORESCENCE FLOW CYTOMETRY / MICROSCOPY)	-	-	%
BAND FORMS (FLUORESCENCE FLOW CYTOMETRY / MICROSCOPY)	-	-	%
METAMYELOCYTE (FLUORESCENCE FLOW CYTOMETRY / MICROSCOPY)	-	-	%
MYELOCYTE (FLUORESCENCE FLOW CYTOMETRY / MICROSCOPY)	-	-	%
PROMYELOCYTE (FLUORESCENCE FLOW CYTOMETRY / MICROSCOPY)	-	-	%
BLAST (FLUORESCENCE FLOW CYTOMETRY / MICROSCOPY)	-	-	%
ATYPICAL CELL	-	-	%
PLATELET COUNT (HYDRO-DYNAMIC FOCUSING IMPEDENCE METHOD/MICROSCOPY)	284	150.00 - 410.00	10 (9)/l
PERIPHERAL SMEAR	RBCs are normocytic normochromic with anisocytosis. WBCs show normal count with monocytosis. Platelets are adequate.		

----- End of Report -----

04-Apr-2024 19:43 PM

Dr.. ROHIT KOTIPALLI

MD, Pathology

Regn.No:194504



MC-2050

Lab Information System

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CANCER HOSPITAL & RESEARCH INSTITUTE

Patient ID	: MR2320948	Reference doctor	: Dr. SENTHIL J RAJAPPA
Patient name	: Mr. RAMA CHANDRA MOHANTY	Category	: CASH
Age / Gender	: 57 Y / Male	Sample collected	: 04-Apr-2024 04:33:42 PM
Visit ID	: HCMR240066183	Sample acknowledged	: 04-Apr-2024 05:24:53 PM
Bed number	:	Reported on	: 05-Apr-2024 08:51:29 AM
Sample no	:	Sample type	: Serum

BIOCHEMISTRY DEPT

TEST NAME (METHOD)	RESULTS	BIOLOGICAL REFERENCE INTERVAL	UNITS
SERUM CREATININE (ENZYMATIC SARCOSINE)	0.48	0.66 - 1.25	mg/dl

----- End of Report -----

05-Apr-2024 08:51 AM

Dr. Sanjay Bagade

MBBS, MD (Biochemistry)
Sr. Consultant, Biochemistry
Regn.No:54888

Note: Please correlate clinically. Kindly discuss if necessary.

Typed By: DD/IS/26 04-Apr-2024 17:04:31 PM

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E-Mail : info@induscancer.com www.induscancer.com



Patient ID	: MR2320948	Reference doctor	: Dr. SENTHIL I RAJAPPA
Patient name	: Mr. RAMA CHANDRA MOHANTY	Category	: CASH
Age / Gender	: 57 Y / Male	Sample collected	: 04-Apr-2024 04:33:42 PM
Visit ID	: HCMR240066183	Sample acknowledged	: 04-Apr-2024 05:24:53 PM
Bed number	:	Reported on	: 05-Apr-2024 08:54:03 AM
Sample no	:	Sample type	: Serum

BIOCHEMISTRY DEPT

TEST NAME (METHOD)	RESULTS	BIOLOGICAL REFERENCE INTERVAL	UNITS
SERUM BILIRUBIN TOTAL (MODIFIED DIAZO)	0.7	0.20 - 1.30	mg/dl

----- End of Report -----

05-Apr-2024 08:54 AM

Dr.. Sanjay Bagade

MBBS, MD (Biochemistry)
Sr. Consultant, Biochemistry
Regn No 84898

NOTE: Please correlate clinically. Re-test if necessary.

Typed By: 000026 : 04-Apr-2024 17:04:25 PM

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BIACHRI/MRE/OP/CASH-3(a)

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Phones: +91-40-2355 1235 / 2355 6655, Fax: +91-40-2354 2120

E-mail: info@induscancer.com • Website : www.induscancer.com



Consultant : _____

OUT PATIENT RECORD

Date of Regn : _____ M.R. No. : _____

Diagnosis : _____

MR2320948 CASH

MR. RAMA CHANDRA MOHANTY (57Y & 0M & 27D / MALE) 28-NOV-2023 / 08:09 AM

SATYABHAMA MOHANTY (MOTHER)

ABHISHEK MOHANTY (SON)

ADDRESS: PLOT NO 350/450, NUASAHI, TAMANDO VILLAGE, KHORDA, ORISSA, INDIA

MOBILE: 9938742412 / 7008758130

CONSULTANTS: DR SENTHIL J RAJAPPA



Dr. T. S. Rao

**ESMO
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CENTRES**



of integrated oncology and palliative care services 11-2013-01-08

Quality matters in cancer care

DATE & TIME

DOCTORS NOTES

13-3-24

C/S/B Dr. Sanyal

cl severe pain in the
perineal region

DIG: P/A - Soft
stomach - Healthy
Perianal region - small
nodule ⊕.

Adv:

- Pain & palliative team
opinion

PB

DATE & TIME

DOCTORS NOTES

Seen by Dr. Sanyal
11/10/24

(small nodule)

ca. Rectum
(stage 2)

↓
soft 1st

↓
liver metast

↓
an chondrosarcoma
+ SPRT + live bag

cl can act
perianal nodule
marking the / lining
the of can

(2+8/10)

Hand / Act Indian

DM

DATE & TIME

DOCTORS NOTES

CELEBRINA DAY

1500mg + 2000

of water (lure)

(twice daily)

[two sides]

see

PER G:

Working hernia ⊕

No sacral ill def-ns

 working? for G
 ↔

Liver lesion ⊕

Continue chemo to 8 cycles

DATE & TIME

DOCTORS NOTES

5/4/24

C7 Capex / Rm

C7 Capex L

2-2

Discharge gel

PAIN / PAINFUL MED.

Gly Spramectol / Adult

- patient reviewed
- on buprenorphine 1mg/1h
change every 7 days.
- Go downsize and
decreased activity
- sleep led; urinary
retention.
- appetite ok
- Bowel & Bladder (v)

Adv

1) Tab. MORPHINE 10mg (1/2)

 1/2 - 1/2 - 1/2 - 1
 6am 12pm 6pm 10pm
1/2 tab 60s.
pain.26/4/24
10:35 AMR/A
week

18/3/24

DATE & TIME	DOCTORS NOTES
	2) Tab. PERINORM 10mg ^{BD} 1 — 1 x 3 days 3am 8pm
	3) Tab. SUPITRUST-8 1tab ^{BD} 1 — 1 x 5 days 9am 8pm
	4) Tab. DULCOTREX (2tabs) 4/4 x — x — x
	5) Sy. Chemopain plus 3am (605)
	6) Tab. MORESADA-RT 400/10 — 9pm
	7) Sitz bath Donut pillow
	8) Cremagel 'H' ointment 1/4
	9) Cremagel ointment 1/4
	J. Anil

no - one
finger
referred

DATE & TIME	DOCTORS NOTES
27/4/24	Cp CapcOs 8.11.7000/2.72
28/4/24	oligomet Retro + liver (sart down)
29-4-24	C/S/B Dr. T.S. Rao c/o severe pain in the perineal region O/E: 3x3cms perineal nodule ⊕ ? Abscess Adv: - USG Perineal region & FNAC from the nodule (Please call Dr. Sanyal)

Dr.

DATE & TIME

DOCTORS NOTES

29/4/24

Radiology Note
 US guided FNA done from
 peri-anal nodules

Dr. Rakshi

Sub-urethral (US)

2/5/24
 11:10 AM

C29-2546

↓
 FNA from Peri-Anal
 lesion - Positive for
 malignancy

PETCT
 (March '24)

→ Soft tissue thickening
 in Pre-Sacral region

→ Complete Resolution
 of Liver lesion

Adv

- Consult Dr. Prathap
 Reddy

Prathap Reddy
 Cancer Surgeon

DATE & TIME

DOCTORS NOTES

2/5/2024

Stable history today.

Follow up

HPE: +ve for malignancy.

Advt

Palliative RT.

Dr. Rakshi

2/5/24

FNA from perianal lesion -
 Metastatic deposits confirmed

Advice

MRI abd and pelvis with contrast

Dr. Rakshi

101

Dr. Rakshi

3/5/24

CPB Dr. Prathap

(Hans. Pallavi)

keloid on rectum.

- 8/10 - 12 Capex /
 Romipha

DATE & TIME

DOCTORS NOTES

- Clo. of down personal pain
- Not controlled : morphine
- Could not lie down
flat for mkt

Adv

- Admission on
palliative care ward
for supportive care
- Analgesic titration
E IV fentanyl
- Continue other
medication
- IV fluids - 500

for
 June 1.

DATE & TIME

DOCTORS NOTES

Pain & Palliative Medicine

PPM Clinic No: 957/2024

Consultant Dr: *praveen*

DATE & TIME

DOCTORS NOTES

9/5/24

SIB Dr. Rakesh

During admission
patient catheterised

Adv

- To continue

Cap Unimax (0.4mg)

X - X - ①

R

10/5/24

TBM opinion: palliative
radiotherapy

Dr. contr. Radiation Oncology

11/5/24

4

DATE & TIME

DOCTORS NOTES

15/5/24

also to Reclaim

ongoing pain RT

- for the visit
- Reclaim pain well controlled
E. Tintanyl 30mg tld
- Sleep OK
- Bowels: stone bag intake (+)

Adv.

1. TINTANYL 30mg tld
change every 22hrs
2. TINTANYL 1000 POP
30mg (SOS) for severe
pain
3. SYPRIMAFAN plus
30mg HS
4. T. MORPHINE 30mg HS

To register with speech hospice for
medications only.

L
15/5

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Quality matters in cancer care

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Rad Accession No : RDB27872713

MRN No : MR2320948

Name : Mohanty Rama Chandra

Age/Gender : 57 Years and 6 Months / M

Consultant :

Category : CS

MRI WHOLE ABDOMEN of 04.05.2024:

Technique:

T1W, T2W, STIR Sagittals T2W STIR axial and coronal imaging.

Findings:

Known case of Carcinoma Rectum, Status post op & post APR.

*Ill-defined heterogeneous T2W/STIR intermediate signal intensity lesion showing restricted diffusion noted involving perianal region including skin & extending superiorly into ischio-anal, precoccygeal regions. Minimal collection noted along the superior aspect in presacral region of pelvis (S4 level). It is closely abutting & indenting the prostate with loss of fat planes. It measures 2.5 x 3.2 x 6 cm – Recurrence.

*Few small volume inguinal lymphnodes noted measuring upto 6 mm in SAD – Non specific.

*Colostomy defect in left iliac fossa with parastomal hernia.

*Umbilical hernia noted.

Liver is normal in size and contour. Parenchyma signal intensities are normal. No focal lesion. No intrahepatic biliary dilatation. Hepatic & Portal veins are normal.

Gallbladder is moderately distended with normal wall thickness. No focal lesion in gall bladder lumen.

Pancreas - Head, body & tail of the pancreas shows normal in size and contour.

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Rad Accession No : RDB27872713

MRN No : MR2320948

Name : Mohanty Rama Chandra

Age/Gender : 57 Years and 6 Months / M

Consultant :

Category : CS

Parenchymal signal intensities are normal. No focal lesion. No evidence of ductal dilatation. Peripancreatic fat planes are normal.

Spleen is normal in size, contour and parenchymal signal intensity. No focal lesion.

Adrenal glands are normal.

Both kidneys are normal in size and contour. Parenchymal signal intensities are normal. No evidence of hydronephrosis on either sides.

Urinary bladder is moderately distended with normal wall thickness. No filling defect in the urinary bladder.

No free fluid in the abdomen / pelvis.

IMPRESSION:

Known case of Carcinoma Rectum, Status post op & post APR.

*Ill-defined heterogeneous T2W/STIR intermediate signal intensity lesion showing restricted diffusion noted involving perianal region including skin & extending superiorly into ischio-anal, precoccygeal regions. Minimal collection noted along the superior aspect in presacral region of pelvis (S4 level). It is closely abutting & indenting the prostate with loss of fat planes. It measures 2.5 x 3.2 x 6 cm – Recurrence.

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*Umbilical hernia

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CENTRES

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Rad Accession No : RDB27872713

MRN No : MR2320948

Name : Mohanty Rama Chandra

Age/Gender : 57 Years and 6 Months / M

Consultant :

Category : CS

Dr. Arvind K Reddy

Report Approved/Verified Date & Time

06.05.2024 15:34



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BIACHRI/MRF/DS-16

DISCHARGE SUMMARY

Patient name	: Mr. RAMA CHANDRA MOHANTY	Patient ID	: MR2320948
Visit ID	: IPMR240014073	Patient category	: MED-ASST.
Age / Gender	: 57 Y / Male	Bed / Nursing station	: ROOM 7-2-NON A/C SHARING
Admission date	: 03-05-2024 4:31PM	Discharge date	: 06-05-2024

Consultant(s)

Dr PRANEETH S M.D.,CCEPC(Palliative Care) D.M(Onco-Anaesthesia)

Final Diagnosis: Carcinoma Rectum, s/p APR pT3N2b → Stage IV (oligometases)

Case History: The patient presented to the radiation oncology outpatient department with complaints of severe perianal pain. He was referred to pain and palliative clinic for analgesic titration.

Summary of Treatment:

On admission, the patient had a pain score of 8-9/10 at the perianal region, sharp and dragging, had difficulty in ambulation and lying down. Local examination did not reveal any positive findings. He was started on inf Fentanyl along with other supportive medications. During the course of stay he had urinary retention and hematuria. Foleys catheterization done and uro-oncology opinion sought, and advice followed. MRI abdomen+pelvis was done as advised by the radiation oncologist to rule out any disease progression. Pain was well controlled gradually and equivalent dose of fentanyl t/d patch was placed. Patch instructions explained and was discharged with appropriate home care measures.

Condition of Discharge: stable

Discharge Advice:

1. Fentanyl 50mcg t/d to change every 72 hours
2. Fentanyl lozenges 200mcg to rub against buccal mucosa in case of severe pain
3. Tab Skebazor 1 tab at 8pm for 5days and stop
4. Tab Pan 40mg once a day before breakfast
5. Tab Moregaba 300mg at night
6. Syr Cremaffin plus 30ml at night
7. Tab Nifas 100mg twice a day for 5days
8. Tab Urimax 0.4mg at night for 15days

Plenty of oral fluids

To review with USG abd+pelvis, urine c/s report in uro-oncology OPD

To review with MRI Report in Radiation oncology OPD.

Report Immediately to the hospital in case of Fever/ Diarrhea/ Excessive Vomiting/ Breathlessness. In case of Emergency contact Ph: 040-2355 1235. Emergency Room (Ext - 2499)

Discharge summary received by
Signature:

Name: Abhishek Mohanty
Relation: son

7008758130

Registrar / Consultant

Signature: Dr. Praneeth S. Suvvari

Name: Dr. Praneeth S. Suvvari
M.D. (Palliative Care)
CCEPC (Palliative Care)
D.M. (Onco-Anaesthesia)
American Cancer Society
Basavarakam Indo-American Cancer Hospital & Research Institute
Road No. 10, Banjara Hills, Hyderabad-500 034, T.S.



BASAVATARAKAM INDO-AMERICAN CANCER HOSPITAL & RESEARCH INSTITUTE

Promoted by Smt Nandamuri Basavataraka Ramarao Memorial Cancer Foundation
& Indo-American Cancer Organisation

Road No 10, Banjara Hills, Hyderabad 500034, India

Phones : 91-40-2355 1235/2355 6655, Fax : 91-40-2354 2120

E-Mail : info@induscancer.com www.induscancer.com



Patient ID	: MR2320948	Reference doctor	: Dr. SUBRAMANYESHWAR
Patient name	: Mr. RAMA CHANDRA MOHANTY		: KAO 1
Age / Gender	: 57 Y / Male	Category	: Cash
Visit ID	: DGMR240058203	Sample collected	: 29-Apr-2024 03:30:02 PM
Bed number	:	Sample acknowledged	: 29-Apr-2024 05:37:49 PM
Sample no	:	Reported on	: 02-May-2024 10:49:01 AM
		Sample type	: FNAC

CYTOLOGY

C24-2546

SPECIMEN :

FNAC from Perianal lesion.

MICROSCOPIC FINDINGS :

Smears show clusters of cells showing pleomorphic round to oval pleomorphic nucleus moderate amount of cytoplasm.

IMPRESSION :

FNAC from Perianal lesion :

Positive for malignancy.

Suggestive of metastatic deposits in a known case of carcinoma rectum.

----- End of Report -----

Handwritten signature

02-May-2024 10:49 AM

Dr. SUSEELA K

MBBS, MD
HOD Pathology
Regn.No-59285

Note: Please correlate clinically. Kindly discuss if necessary.

Typed By 01860 : 30-Apr-2024 14:04:09 PM

This report relates to the specimen / material submitted to the laboratory

The results that are shown in bold indicate as critical result

Dr. Praneeth Suvvari

M.D., CCEPC

D.M. (Onco-Anaesthesia) (AIIMS), NFPM (Palliative Medicine)

Fellowship in Advanced Pain Management

Consultant Oncoanaesthesia

Pain & Palliative Medicine

E-mail : praneethsuv@gmail.com

**Basavatarakam Indo-American
Cancer Hospital & Research Institute**

Promoted by Smt. N.B.T.R. Memorial Cancer
Foundation & Indo-American Cancer Organisation

BIACHRI/MRF/DP-6



Name : *Ramra Chandra Mohanty*

MR No. : *2320948*

Date : *3/5/24*

Also of Co. Referred

→ S/P - C7 Capex / Poni

→ S/P - Radiotherapy to Hepatic lesion

Cl of severe pruritic pain

- Not Controlled on oral Morphine long 4hly.

- Unable to pass urine freely.

- Unable to eat orally - 3 days

Adv

*- Admission in palliative Care ward
for Analgesic titration & IV fentanyl
supportive care.*

Pain & Symptom Control

attached.



Quality matters in Cancer Care

Dr. Praneeth Suvvari
M.D., CCEPC, (Palliative Care),
D.M. (Onco - Anaesthesia) (AIIMS),
Fellowship in Advanced Pain Management
Consultant Oncoanaesthesia
Pain & Palliative Medicine
Reg No: APMC/FMR/77093
Basavatarakam Indo - American
Cancer Hospital & Research Institute
Road No. 10, Banjara Hills, Hyderabad-500 034, T.S